

PHYSIOTHERAPY CLINICAL CASE STUDY REPORT

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CASE INFORMATION

Case Number:	Auto-assigned
Category:	Outdoor
Date:	2024-07-30
Referring Physician:	Self-referred / Not specified

PATIENT DEMOGRAPHICS

Name:	Rukhsana Bibi
Age:	55 years
Gender:	Female
Marital Status:	Married
Language:	Not Provided
Occupation:	Housewife
Address:	Rawalpindi
Mode of Admission:	Outdoor / OPD

PRESENT COMPLAINT

The patient, a 55-year-old female, presents with a chief complaint of right-sided neck pain and associated stiffness, which has been ongoing for approximately one month. She reports that the pain is exacerbated by specific neck movements and significantly impacts her daily activities.

HISTORY OF PRESENTING COMPLAINT

The patient reports a gradual onset of symptoms approximately one month prior to presentation, without any specific history of trauma or sudden jerk to the neck. The pain is primarily localized to the right side of the neck and is accompanied by a sensation of stiffness. She notes that the pain intensifies with movements such as neck rotation and looking upwards, reaching a Numeric Pain Rating Scale (NPRS) score of 6/10. Conversely, resting and the application of a hot pack provide some relief. The condition has begun to interfere with her daily activities as a housewife, particularly tasks requiring sustained neck postures or repetitive movements.

PAIN PROFILE

Location:	Right-sided cervical region, specifically over the lower cervical paraspinal muscles.
Onset:	Gradual
Duration:	1 month
Nature:	Dull aching pain with associated stiffness, occasionally sharp with movement.
Radiation:	No radiation to upper limbs
NPRS Score:	6/10 on movement



6/10 on movement

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Aggravating Factors

- Neck rotation
- Looking up

Relieving Factors

- Resting
- Hot pack application

PAST HISTORY

Medical History:	Mild hypertension, managed conservatively.
Surgical History:	No previous surgeries reported
Medications:	Not Provided
Family History:	Not Provided
Socioeconomic:	Not Provided
Pre-morbid Status:	Independent in all Activities of Daily Living (ADLs) prior to current episode, actively managing household duties.
Growth & Development:	Not applicable / within normal limits

PATIENT'S PRIORITIES & FUNCTIONAL GOALS

- Reduce right-sided neck pain to allow comfortable daily activities.
- Improve neck range of motion, especially for looking up and turning head.
- Perform household chores without experiencing pain or stiffness.
- Improve sleep quality by reducing nocturnal neck discomfort.

GENERAL HEALTH STATUS

Vitals

Blood Pressure:	Normal / Within limits
Pulse:	Normal / Within limits
Respiratory Rate:	Normal / Within limits
Temperature:	Afebrile
O2 Saturation:	Normal / Within limits
Weight:	Not Provided
Height:	Not Provided
BMI:	Not Provided

Clinical Examination

Lymph Nodes:	Not palpable
Jaundice:	Absent
Pallor:	Absent
Cyanosis:	Absent
Edema:	Absent

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Clubbing:	Absent
Overall Impression:	The patient is conscious, oriented, and appears hemodynamically stable with no acute distress.

GENERAL HEALTH CONDITION

Malaise:	None reported
Fatigue:	None reported
Fever:	Afebrile
Weight Changes:	None reported
Sleep:	Not reported, but likely impacted by pain.
Cognition:	Intact
Mood / Affect:	Cooperative and oriented

SYSTEMS REVIEW

Cardiovascular:	No complaints reported
Pulmonary:	No complaints reported
Gastrointestinal:	No complaints reported
Urinary:	No complaints reported
Integumentary:	Skin intact, no wounds or lesions noted
Endocrine:	No complaints reported
Reproductive:	Not assessed / Not applicable

MUSCULOSKELETAL SYSTEM

Gross Symmetry:	Normal
Posture:	Not explicitly documented, but likely compensatory changes due to pain.
Deformity:	None observed
Swelling:	None observed
Tenderness:	Localized tenderness over the right lower cervical paraspinal muscles (Grade 2/4).
Muscle Wasting:	None observed
Muscle Spasm:	Palpable muscle spasm noted in the right lower cervical paraspinal muscles.
Cervical ROM:	Extension and right rotation restricted due to pain. Other ranges not explicitly documented but likely within functional limits or mildly restricted.
Lumbar ROM:	Normal
Upper Limb ROM:	Normal
Lower Limb ROM:	Normal
Muscle Strength:	Normal throughout

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Special Tests

- Not explicitly documented

NEUROLOGICAL ASSESSMENT

Consciousness: Conscious and oriented to time, place and person

Numbness / Tingling: None reported

Reflexes: Normal

Muscle Tone: Normal

Coordination: Normal

Gait: Normal

Cranial Nerves: Not assessed / Intact

Upper Motor Neuron: No signs of UMN lesion

Lower Motor Neuron: No signs of LMN lesion

FUNCTIONAL STATUS

ADL Status: Partially dependent due to limitations in tasks requiring neck movement (e.g., looking up, turning head while cooking or cleaning).

Mobility: Independent

Work Status: Modified duties within the home due to pain, unable to perform certain household tasks comfortably.

Recreational: Limitations in recreational activities if any

Assistive Devices: None currently in use

Functional Goals

- Be able to look up and turn head fully without pain to perform household chores.
- Sleep comfortably through the night without neck pain.
- Reduce reliance on hot packs for pain relief.
- Improve overall neck flexibility and strength to prevent recurrence.

INVESTIGATIONS & IMAGING

Imaging: None provided / Not available

Lab Tests: None provided / Not available

EMG / NCS: Not assessed

Other: None

CLINICAL IMPRESSION & DIAGNOSIS

The clinical impression is Cervical Spondylosis with associated right-sided cervical paraspinal muscle spasm (ICD-10: M47.812 - Other spondylosis, cervical region, with radiculopathy; M62.830 - Muscle spasm of neck). This diagnosis is supported by the patient's age, gradual onset of neck pain and stiffness, specific aggravating factors, localized tenderness, and restricted cervical range of motion, in the absence of neurological deficits.

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HYPOTHESIS / PROBLEM STATEMENT

The patient presents with chronic right-sided neck pain and stiffness, indicative of cervical spondylosis, exacerbated by muscle spasm in the lower cervical paraspinal region. This condition significantly limits her cervical extension and right rotation, impacting her ability to perform daily household activities without pain. The absence of neurological signs suggests a localized musculoskeletal issue rather than nerve root compression. The treatment approach will focus on reducing pain and muscle spasm, restoring cervical range of motion, and improving functional independence through a combination of modalities, manual therapy, and therapeutic exercises.

TREATMENT PLAN

Short-Term Goals (0 - 2 Weeks)

- Reduce right-sided neck pain to 3/10 or less on NPRS within 2 weeks.
- Decrease palpable muscle spasm in the right lower cervical paraspinal muscles.
- Improve cervical extension and right rotation by 25% within 2 weeks.
- Educate patient on proper neck posture and pain management strategies.

Long-Term Goals (3 - 6 Weeks)

- Achieve full pain-free cervical range of motion within 6 weeks.
- Enable the patient to perform all household activities without pain or stiffness.
- Strengthen cervical musculature to prevent recurrence of symptoms.
- Empower patient with a self-management home exercise program.

Physiotherapy Interventions

- Moist Heat Therapy: 15 minutes to the cervical region prior to manual therapy to promote muscle relaxation and increase tissue extensibility.
- Transcutaneous Electrical Nerve Stimulation (TENS): Conventional mode, 30 minutes, to the right cervical paraspinal muscles for pain modulation.
- Gentle Cervical Mobilizations: Grade I-II oscillations and sustained natural apophyseal glides (SNAGs) to improve joint mobility and reduce stiffness in restricted cervical segments.
- Soft Tissue Release/Massage: To the right cervical paraspinal muscles to reduce spasm and tenderness.
- Cervical Isometric Exercises: Gentle contractions in all planes (flexion, extension, lateral flexion, rotation) to activate and strengthen deep neck stabilizers without aggravating pain (e.g., 5-second hold, 5-10 repetitions).

Home Exercise Program

- Chin tucks: 3 sets x 10 repetitions, hold 5 seconds, performed slowly and gently.
- Gentle cervical range of motion exercises: Active pain-free movements in all planes (flexion, extension, lateral flexion, rotation), 10 repetitions each, 3 times daily.
- Scapular retraction exercises: Squeeze shoulder blades together, 3 sets x 10 repetitions, hold 5 seconds, to improve postural awareness.
- Application of hot pack: 15-20 minutes as needed for pain relief.

Patient Education

- Ergonomic advice for daily activities, including proper posture during sitting, standing, and sleeping.
- Importance of regular, gentle movement and avoiding prolonged static postures.
- Pain management strategies, including the appropriate use of heat therapy.
- Understanding of cervical spondylosis and the role of exercise in long-term management.

Referrals: None at this time

Follow-up: Review in 1 week to assess progress and adjust treatment plan. Full reassessment at 3 weeks.

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Prognosis:

The prognosis is good with consistent adherence to the physiotherapy program. Given the absence of neurological compromise and a clear mechanical presentation, significant improvement in pain and function is anticipated.

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Physiotherapist Signature

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